Docetaxel alone for previously treated non-small-cell lung cancer

Treatment scope · NCCP 00203 NCCP 00203a: locally advanced or metastatic non-small-cell lung cancer after failure of prior chemotherapy. This is docetaxel alone; a platinum or other partner needs its own regimen.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Docetaxel · lung supportive care

Active medicines: Docetaxel

Emetogenic risk: low

Medicine	Dose / route	When
Dexamethasone	8 mg orally twice daily, unless contraindicated	For three days, starting the day before each docetaxel infusion National Cancer Control Programme (NCCP) requires a minimum of three pre-treatment doses. If doses were missed, contact the team; intravenous (IV) replacement is a consultant decision. Reconcile with any other steroid prescription.
Ondansetron	8 mg orally	On treatment day Review QT, constipation and hepatic dosing; reconcile any steroid already prescribed
Metoclopramide	10 mg orally up to three times daily as needed	Subsequent days: duration to be confirmed Assess contraindications, renal function and interactions

Additional phase-specific support

- Dexamethasone 8 mg orally twice daily for three days starting the day before docetaxel, unless contraindicated; NCCP requires at least three pre-treatment doses. Missed doses require consultant review; 20 mg IV immediately before infusion is a discretionary alternative.
- Avoid duplicate corticosteroid prescriptions. granulocyte colony-stimulating factor (white-cell growth support) (G-CSF) may be used after individual risk assessment. Monitor fluid retention and neuropathy.
- Low emetic risk: prevention and rescue medicines remain separate from reaction premedication. Confirm individual suitability, interactions and duration; this reference is not a prescription.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00203 v7 · p3 · NCCP NCIS medical-oncology antiemetics V8, low risk

Sources checked 2026-11-02 · SACTCheck v0.84.29 · Regimen-level drafts; prescriber and oncology-pharmacy verification required